

Regulating Insurer Use of Genetic Information

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Without adequate legislation and enforcement, non-health insurance companies may discriminate against individuals on the basis of their genetic testing status or results. The Genetic Information Nondiscrimination Act (GINA) is a federal act that prohibits health insurers and employers from determining individuals' insurability, renewability, premiums, benefits, and conditions on the basis of genetic information.¹ However, the law's protections do not extend to other insurers, such as life, long-term care, and disability insurance. Here, discrimination may take the form of higher premium rates, limitations on coverage, the denial or cancellation of coverage, or the imposition of additional policy conditions.

As technology rapidly advances, it is important to draft legislation that anticipates and blocks avenues for non-health insurers to discriminate on the basis of genetic information. It is also critical to clarify whether the public meaning of a word or phrase aligns with its intended effect under law because every statutory term that legislators include or exclude can shift the balance between insurer capabilities and individual rights.

This white paper aims to provide legislators with the legal tools necessary to draft legislation in a way that aligns with their intentions and constituent expectations. More specifically, it outlines key elements to consider when: (I) identifying the relevant insurer types, (II) preventing unfair discrimination between individuals of the same insurance class, (III) requiring or requesting informed consent, (IV) defining genetic test and genetic information, and (V) identifying the permissible range of insurer actions.

I. TYPE OF INSURER

Key Provision: Ensure that policy accounts for an adequate variety of insurance companies.

Federal law, under GINA, only applies to health insurers and employers. It does not prevent life, long-term care, disability, and other insurers from using genetic data to differentiate individuals' insurance policies. Thus, before passing legislation on genetic testing, state legislators should consider which insurers could, and should, be impacted by the terms of a proposed statute. Here are two factors to consider:

¹ *Genetic Discrimination*, NATIONAL HUMAN GENOME RESEARCH INSTITUTE, <https://www.genome.gov/about-genomics/policy-issues/Genetic-Discrimination>.

A. Is the statute sufficiently inclusive of the relevant insurer types?

While many states mirror GINA in prohibiting health insurers from using genetic information, fewer restrict life, disability, and long-term care insurers. Florida offers a more comprehensive restriction, asserting that:

In the absence of a diagnosis of a condition related to genetic information, health insurers, life insurers, and long-term care insurers authorized to transact insurance in this state may not cancel, limit, or deny coverage, or establish differentials in premium rates, based on such information.²

Here, due to other state law provisions, health insurers include disability insurers.³ As such, this statute casts a broad net, forbidding insurer types beyond health insurance from using genetic information to differentiate between individuals of the same class.

Other state statutes only reference certain insurer types. For example, Idaho specifies that:

No person shall discriminate on the basis of a genetic test or private genetic information, as those terms are defined in section 39-8302, Idaho Code, in the issuance of coverage, or the fixing of rates, terms or conditions, for any policy or contract of disability insurance or any health benefit plan.⁴

Other Idaho provisions confirm that health benefit plans do not include disability income, long-term care, automobile medical payment insurance, among others.⁵ Thus, explicitly stating that the disability insurers and health benefit plans shall not unfairly discriminate on the basis of genetic information implies that it is permissible for the other insurer types not mentioned, like long-term care insurers, to discriminate.

If a state is interested in preventing all or most insurers from discriminating on the basis of genetic information, it may be beneficial to craft the statute using inclusive language and then supplement the inclusive language with any specific, desired exceptions.

B. Does the statute consider entities that might be interested in using genetic information in the future?

Historically, mortgage companies, real estate sellers, and other insurers outside the healthcare industry have not used individuals' genetic information to determine their insurability, premium rates, benefits, and conditions.⁶ But these businesses may be interested in using genetic information in the future, especially with new genetic technologies.⁷ State statutes should take initiative and account for all potentially interested entities to prevent any undesired differentiation on the basis of genetic information in the future.

² FLA. STAT. § 627.4301(2)(a) (2020).

³ See FLA. STAT. § 627.4301 (2020).

⁴ IDAHO CODE § 41-1313(3) (2006) (emphasis added).

⁵ IDAHO CODE § 41-4703(13) (2013).

⁶ Mark A. Rothstein & Laura Rothstein, *The Use of Genetic Information in Real Property Transactions*, in TR. & EST. L., AM. BAR ASS'N, at 1, (Univ. of Louisville L. Sch., Legal Stud. Rsch. Paper Ser. No. 2017-7, 2017).

⁷ Michelle N. Meyer et. al., *Potential corporate uses of polygenic indexes: Starting a conversation about the associated ethics and policy issues*, 111 AM. J. HUM. GENETICS 833, 835 (2024).

II. UNFAIR DISCRIMINATION

Key Provision: Meaningfully limit insurer use of genetic information.

Typically, statutes governing insurer use of genetic data fall into one of three categories. They either prohibit insurers from using genetic information to differentiate between individuals of the same insurance class, allow insurers to do so, or forbid insurers from doing so unless differentiation is actuarially justified.

However, in the latter two categories, state insurance commissioners are often deferential to insurers. Thus, despite complexities and nuances in genetic risk prediction, insurers may easily be able to satisfy the actuarial justification requirement and lawfully adjust an individual's insurance policy. Yet, in some instances, lawmakers may begin with the intention of barring insurer use of genetic information but may nonetheless be encouraged by insurers to allow for differentiation on the basis of actuarial justification.

It remains incredibly difficult for consumers to ascertain what evidence went into insurers' determinations of actuarial justification. In addition, actuarial justification laws add little protection beyond the status quo and may confuse constituents who may interpret them as offering more protection than they actually do. Thus, statutory language must be examined to identify if, how, and when insurers may lawfully differentiate between individuals of equal risk. Here are some questions to consider:

A. Does the statute permit coverage differentiation as long as such differentiation is actuarially justified?

State laws often assert that insurers can only use genetic information to differentiate rates between individuals of the same insurance class if the action is actuarially justified. Generally, an action is actuarially justified if the difference in coverage aligns with the difference in cost the insurer incurs.⁸ For example, if an insurer raises a smoker's premium rates so that they are higher than the premium rates of a nonsmoker in the same insurance class, this action would be actuarially justified because there is ample evidence that smoking increases the likelihood of developing certain serious diseases and thereby increases insurer costs. However, differentiating coverage rates of individuals in the same insurance class on the basis of eye color would not be actuarially justified because eye color, to date, is not linked to any likely predisposition of disease or disability that would plausibly raise costs to insurers.

However, because they have the power to write their own laws, states vary as to what actions are classified as actuarially justified. For example, Maryland allows insurers to differentiate coverage rates on the basis of sex, physical handicap, disability, and previous and future travels that create a material difference in risk to consumers, and thus, also to insurers.⁹ On the other hand, Kansas does not identify specific instances when differentiation on the basis of genetic information is permissible, but rather forbids life, disability income, and long-term care insurers from "provid[ing] for rates or any other aspect of coverage that is not reasonably related to the

⁸ Anya E.R. Prince, *Insurance Risk Classification in an Era of Genomics: Is A Rational Discrimination Policy Rational?*, 96 Neb. L. Rev. 624, 630–31 (2017).

⁹ See MD. CODE, INS. § 27–208 (2016).

risk involved.”¹⁰ In other words, if an individual’s genetic information can reasonably be perceived to influence insurers’ risk, insurers in Kansas may differentiate coverage rates, benefits, conditions, and the like.

B. Does the statute include the phrase “unfair discrimination”?

Insurers interpret “unfair discrimination” as treating an individual within the same insurance class differently because of his or her genetic information. Here, a class is a group of people an insurance company has determined share a similar risk level. Consequently, insurers might argue that they can use genetic information to place an individual in a class but cannot use genetic data to differentiate between people of the same class. In other words, as long as an insurer has some actuarial justification or evidence to place an individual in a certain risk class, they can charge that customer a higher premium.

However, the general public may interpret the phrase “unfair discrimination” differently than insurers. For instance, the general public may define unfair discrimination as treating an individual differently because of his or her race, color, religion, sex, disability, or genetic information.¹¹ As such, individuals might mistakenly believe that the statute prohibits insurers from using genetic testing results at all, including placing them in a particular insurance class. The public may also misunderstand the impact of a statute that prohibits unfair discrimination because these statutes generally allow insurers to use genetic information for actuarially justified actions.

C. Does the statute include the phrase “solely based on” or “on the basis of”?

The phrases “solely based on” or “on the basis of” often allow insurers to use a genetic trait to differentiate between individuals *as long as* the action is actuarially justified. For example, Arizona law asserts that:

The rejection of an application or the determining of rates, terms or conditions of a life or disability insurance contract on the basis of a genetic condition, developmental delay or developmental disability constitutes unfair discrimination, unless the applicant's medical condition and history and either claims experience or actuarial projections establish that substantial differences in claims are likely to result from the genetic condition, developmental delay or developmental disability.¹²

In short, if an insurer in Arizona can establish that a genetic condition or developmental disability creates a significant increase in risk, then the insurer can adjust the individual’s policy rates, benefits, and conditions to reflect that risk. Given the actuarial justification exception, the statutory language “solely based on” or “on the basis of” might not provide the strength of protection that the general public may expect.

¹⁰ KAN. STAT. § 40–2259(d)(2) (2010).

¹¹ See Discrimination, harassment, and retaliation, U.S. GEN. SERVS. ADMIN. (July 23, 2025) <https://www.usa.gov/job-discrimination-harassment>.

¹² ARIZ. REV. STAT. § 20–448(E) (2019) (emphasis added).

III. INFORMED CONSENT

Key Provision: Provide more protections than only consent.

Many states require insurers to receive an individual's informed consent before using his or her genetic information. Although the precise definition varies by state, informed consent is the process of educating an individual about the potential uses of his or her genetic testing results, followed by the retrieval of the individual's permission to do so. Insurers can obtain informed consent either *directly* from individuals or *indirectly* through direct-to-consumer companies or healthcare providers.

Given the multitude of avenues for satisfying the consent requirement, state legislators interested in strengthening informed consent laws should consider the following factors when reviewing or passing legislation on genetic testing:

A. Does the statute address insurers' ability to satisfy the informed consent requirement through individuals?

If the law allows insurers to request individuals' genetic testing results or participation status during the insurance application process, individuals can choose to either accept or deny this request. For example, Iowa declares that "A person shall not obtain genetic information or samples for genetic testing without first obtaining informed and written consent from the individual or the individual's authorized representative."¹³ However, if an individual denies this request, the insurer may be able to upcharge their policy, limit benefits, impose additional conditions, or reject their application altogether. Given that insurers ultimately decide who has access to coverage, individuals may feel that they must consent in order to receive an insurance policy. Thus, although receiving consent will keep the client informed of what privacy rights they are relinquishing, prohibiting insurers from using genetic information to craft coverage policies will ultimately offer the strongest protection of personal client data.

B. Does the statute account for insurers' ability to obtain informed consent through direct-to-consumer companies?

Direct-to-consumer companies are businesses that sell products and services to consumers online,¹⁴ among which include genetic testing. Since the Health Insurance Portability and Accountability Act (HIPAA) does not regulate direct-to-consumer companies, there is nothing preventing these companies from using or selling clients' genetic data to life, disability, and long-term care insurers under federal law. However, states and direct-to-consumer companies' privacy policies may restrict the use and sale of client genetic information.¹⁵

¹³ IOWA CODE § 729.6(3)(a) (2023) (emphasis added).

¹⁴ Richard Kestenbaum, *How Direct-To-Consumer Companies Succeed—And Why Many Fail*, FORBES (Sept. 21, 2020), <https://www.forbes.com/sites/richardkestenbaum/2020/09/21/how-direct-to-consumer-companies-succeed-and-why-many-fail/>.

¹⁵ Anya E.R. Prince & Kayte Spector-Bagdady, *Protecting Privacy When Genetic Databases Are Commercialized*, J. of the American Medical Association, 333, 665–66 (2025).

For example, Maine asserts that:

In connection with the issuance, withholding, extension or renewal of an insurance policy for life . . . disability, [or] long-term care . . . an insurer may not request, require, purchase or use information obtained from an entity providing direct-to-consumer genetic testing without the informed written consent of the individual who has been tested.¹⁶

In short, this statute requires insurers to receive an individual's written, informed consent before accessing his or her genetic information through a direct-to-consumer company. Other states mandate informed consent at additional interactions with direct-to-consumer companies. For example, Virginia requires:

Every direct-to-consumer genetic testing company . . . [to] obtain a consumer's express consent for the collection, use, and disclosure of the consumer's genetic data, including, at a minimum, separate and express consent for each of the following . . . (1) us[ing] . . . genetic data collected through the genetic testing product or service offered to the consumer (2) stor[ing] . . . a consumer's biological sample after the initial testing . . . (4) transferr[ing] . . . the consumer's genetic data or biological sample to a third party other than a service provider . . . (5) marketing or facilitation of marketing based on the consumer's genetic data.¹⁷

Yet even though informed consent is required more frequently in the quoted language from Virginia than in the quoted language from Maine, the same issue arises. If the individual declines or is permitted to revoke consent at any point, both of these statutes fail to prohibit insurers from denying or increasing the rates of that individual's policy. As such, individuals may feel that they have no other option than to provide written consent in order to receive the coverage they need. Thus, although the informed consent requirement may be legally satisfied, consent may not be willing or voluntary.

To address this concern, a 2022 California statute eliminated the informed consent requirement. This statute asserts that a direct-to-consumer company may not "disclose a consumer's genetic data or biological sample to an entity [or to any agent or division of an entity that is] . . . primarily engaged in administering health insurance, life insurance, long-term care insurance, [or] disability insurance."¹⁸ However, insurers can still ask individuals directly about their direct-to-consumer genetic test results, which may pressure individuals to release these results in order to receive the insurance they need. Furthermore, this statute also allows direct-to-consumer companies to disclose clients' genetic data to businesses that offer insurance policies, but whose primary purpose is to provide another type of insurance.

C. Does the statute consider an insurer's ability to obtain access to medical records with authorization?

The Health Insurance Portability and Accountability Act (HIPAA) forbids health providers from sharing genetic test results with non-health insurers without patient authorization.¹⁹ Yet, many life insurance applications²⁰ offer a waiver to comply with HIPAA regulations. Valid waiver

¹⁶ ME. STAT. tit. 24-A, §2159-C(4) (2019) (emphasis added).

¹⁷ VA. CODE § 59.1-596 (2023) (emphasis added).

¹⁸ CAL. CIV. CODE § 56.181(f)(1).

¹⁹ Health Insurance Portability and Accountability Act, 45 C.F.R. § 164.502(a) (West 2024).

²⁰ See <https://hinermangroup.com/wp-content/uploads/2009/05/hipaa-authorizationlog.pdf>; see also <https://home.globelifeinsurance.com/GlobeLifeHome/media/pdfs/ln/R3590C.pdf>.

signatures satisfy the informed consent requirement of many states and often allow life insurers to access, use, and disclose the individual's medical records, which may include genetic information.

However, applicants may feel they have no other choice but to sign the waiver. For example, Liberty National Life Insurance Company's [waiver](#) declares that: "I further understand that if I refuse to sign this authorization to release my complete medical record, LNL may not be able to process my application, or if coverage has been issued, may not be able to process policy claims." As such, the waiver essentially delivers an ultimatum, thereby reducing individual autonomy in important medical and financial decisions.

IV. DEFINITION OF GENETIC TEST

Key Provision: Make sure the definition of genetic test/information is sufficiently comprehensive.

GINA defines genetic information broadly to include the individual's genetic test results, their family members' genetic test results, family medical history, and any participation in genetic services. Although GINA only applies to employers and health insurers, the law's comprehensive language can serve as an example of factors to consider when drafting definitions of genetic testing in broader state laws. Currently, many state definitions of genetic testing and genetic information are not as comprehensive as GINA's as they relate to life, disability, and long-term care insurance.

Before passing legislation on genetic testing, state legislatures should assess their current definition of genetic test and genetic information to avoid unintended consequences. Here are some factors to consider:

A. Does the definition of genetic information include the genetic data of family members?

Some states have limited genetic testing to include only the individual's participation in genetic testing and genetic test results.²¹ For example, in Arizona, genetic testing is defined as "an analysis of an individual's DNA, gene products or chromosomes that indicates a propensity for or susceptibility to illness, disease, impairment or other disorders."²² The additional definitions of genetic condition and gene products also do not reference family members.²³ By default, Arizona may allow insurance companies to use family medical history, family members' participation in genetic services, or family members' genetic test results to infer the individual's genetic makeup for purposes of creating or renewing an insurance policy.

Other states appear to address this loophole. For example, Maine defines genetic information as "the information concerning genes, gene products or inherited characteristics that may be obtained from an individual or family member."²⁴ By including the use of family members' genes in the definition of genetic information, this statute has eliminated one alternative avenue

²¹ ARIZ. REV. STAT. § 20-448(M)(6); MASS. GEN. LAWS ch. 175, § 120E; 108L.

²² ARIZ. REV. STAT. § 20-448(M)(6).

²³ *Id.*

²⁴ ME. STAT. tit. 24-A, §2159-C(1)(B) (2019).

that insurers may use to calculate an individual's insurability, premium rates, conditions, and benefits.

On the other hand, some states explicitly give insurers permission to use family members' genetic information to determine an individual's insurance policy. For example, New Hampshire asserts that:

A person in the business of providing life, disability income, or long-term care insurance who obtains information with respect to any genetic testing of an individual or a member of the individual's family shall not use that information in writing a type of insurance coverage other than life, disability income, or long-term care insurance.²⁵

This approach allows the specified insurers to draw from a larger pool of genetic information to calculate coverage. However, family members might not be aware that their participation in a genetic test could lead to their genetic information being used to determine their biological family members' life, long-term care, and disability insurance eligibility, premiums, benefits, and conditions.

Policymakers should think carefully about the distinction between family medical history and family members' genetic test results. Both are considered genetic information under GINA.²⁶ Thus, to date, family medical history has been used to determine health insurance coverage.²⁷ But, family medical history is also widely used in non-health insurance underwriting. As a result, a new law prohibiting non-health insurers from using family medical history to estimate insurability, premiums, benefits, or conditions would likely generate a much larger impact than a law forbidding the calculation of non-health insurance coverage, rates, and benefits on the basis of genetic test results alone.

B. Is the definition of genetic testing unnecessarily limited to associations with disease, illness, or disorders?

Some states define genetic tests as only those that are known to be associated with an *increased* risk of disease or disorder.²⁸ For example, Massachusetts defines genetic testing as the analysis of:

Human DNA, RNA, mitochondrial DNA, chromosomes or proteins for the purpose of identifying genes, inherited or acquired genetic abnormalities, or the presence or absence of inherited or acquired characteristics in genetic material, which are associated with a predisposition to disease, illness, impairment or other disease processes.²⁹

But by default, this narrow definition could allow insurers to use variants of a gene whose significance is *unknown*, or variants of a gene known to be associated with a *decreased* risk in developing a disease or disorder.

²⁵ N.H. REV. STAT. § 141-H:5 (emphasis added).

²⁶ Genetic Information Nondiscrimination Act, 42 U.S.C.A. § 2000ff-1(b) (West 2008).

²⁷ Richard K. Linner & Philipp D. Koellinger, *Genetic risk scores in life insurance underwriting*, 81 J. HEALTH ECON. 1, 13 (2022).

²⁸ CAL. INS. CODE § 10147(b) (2000); MD. CODE, INS. § 27-909(a)(5) (2020).

²⁹ MASS. GEN. LAWS ch. 175, § 108I(a) (emphasis added).

C. Does the definition account for future advancements in genetic testing?

Though still being developed, polygenic scores are an emerging category of genetic test results. A polygenic score identifies associations between genetic variants and a given disease or behavior.

In some instances, people have used polygenic scores to *predict* an individual's risk for developing certain diseases.³⁰ For example, life, disability, and long-term care insurers will likely be interested in polygenic scores that measure an individual's risk of cancer, heart attacks, strokes, and other conditions.³¹ A potential concern is that no one will be completely risk free, and "it is not known how different conditions and traits w[ill] be weighted by life," disability, and long-term care insurers.³² However, states whose laws define genetic test as an association with a disease could regulate polygenic scores that indicate individuals' predisposition to disease.

Other polygenic scores are associated with behavioral and social traits.³³ One such polygenic score measures an individual's genetic contributions to their 'risk tolerance' and willingness to engage in risky behaviors, such as driving fast, drinking, and smoking.³⁴ Insurers may also be interested in these traits.

If a state law defines genetic testing as a predisposition to a disease or disorder, the law may not cover these newer types of genetic testing. To avoid unintended consequences, state legislators should define 'genetic test' to broadly account for any relevant advancements that may impact insurer access and use of consumer genetic data.

V. TYPE OF INSURER ACTION

Key Provision: Examine whether legislation addresses all possible ways insurers could utilize genetic information.

There are many different points in time when insurers can use an individual's information to make decisions about that person's coverage.

First, an insurance company strategically markets its services, and any interested person may apply for coverage. The application usually requires individuals to share their personal information with the insurer. Once submitted, the insurance company will review the individual's application to determine if, and at what rate, an insurance policy could and should be offered. Based on this review, the insurer will either reject the application or offer the applicant a policy. If the applicant accepts the insurer's offer, most policies will be effective for a select period of time, subject to potential renewal.

³⁰ Lia Petronio, *Polygenic Scores*, BROAD INSTITUTE, <https://polygeniccores.org/explained/>.

³¹ Richard K. Linnér & Philipp D. Koellinger, *Genetic risk scores in life insurance underwriting*, 81 J. HEALTH ECON. 1, 2 (2022).

³² Tatiane Yanes et al. *Future implications of polygenic risk scores for life insurance underwriting*, 9 NPJ GENOMIC MED. 1, 3 (2024).

³³ Courtney Canter et al. *Scanning the Horizon of Sociogenomics: an Assessment of the Development and Growth of Polygenic Indices for Social and Behavioral Traits*, AJOB EMPIRICAL BIOETHICS 1, 4–5 (2025).

³⁴ Richard K. Linnér et al., *Genome-wide association analyses of risk tolerance and risky behaviors in over 1 million individuals identify hundreds of loci and shared genetic influences*, 51 NATURE GENETICS 245, 258 (2019).

When drafting or reviewing bills on any type of insurer action, state legislators should consider these factors:

A. Does the statute account for insurers' strategic marketing techniques?

Insurers may be incentivized to use genetic information as the basis for where and how to market their services. For example, if experts determine that a particular group of people is more likely to develop a given disease or disorder because of their genetic makeup, insurers may refrain from marketing their services to the communities where these individuals live. Although this strategy might be profitable for insurers, higher risk individuals may find it more difficult to obtain affordable insurance policies than lower risk individuals, resulting in potential marginalization of access if left unregulated and unenforced.

B. Does the statute allow insurers to request genetic test results or participation status to determine coverage?

Some states allow insurers to *request* genetic test results or participation status on coverage application or renewability forms to scaffold coverage rates, benefits, or conditions. For example, California asserts that “No admitted insurer, licensed to issue disability insurance for hospital, medical, and surgical expenses, shall seek information about a person’s genetic characteristics for any nontherapeutic purpose.”³⁵ Here, disability insurers are prohibited from requesting individuals’ genetic information for the purpose of determining coverage premiums. But, by default, any insurer not mentioned in the statute may do so.

After receiving this request for genetic information, individuals can choose to either provide their genetic test results or participation status, or they can decline to respond. However, if an individual declines to answer this prompt, the insurer may respond by declining their request for or the renewal of coverage, increasing the individual’s premium rates, reducing their benefits, or placing additional conditions or restrictions on their policy.

On the other hand, other states explicitly prohibit insurer requests for genetic testing results or participation status. For example, Florida declares that:

Health insurers, life insurers, and long-term care insurers may not require or solicit genetic information, use genetic test results, or consider a person’s decisions or actions relating to genetic testing in any manner for any insurance purpose.³⁶

Other provisions clarify that health insurers include disability insurers under state law.³⁷ Thus, Florida offers one of the most comprehensive bans on insurer solicitation. Floridian insurers may not ask for individual’s genetic testing results or participation status, nor may they use the decision to refrain from voluntarily providing this information as sufficient reason to decline coverage, increase premium rates, reduce benefits, or impose additional conditions.

To maximize the right to protect personal information, state legislatures should consider prohibiting insurers from requesting or requiring genetic test results or participation status in order to calculate individuals’ premiums, benefits, or conditions.

³⁵ CAL. INS. CODE § 10140(c) (2012) (emphasis added).

³⁶ FLA. STAT. § 627.4301(2)(b) (2020) (emphasis added).

³⁷ FLA. STAT. § 624.603 (2003).

CONCLUSION

Statutory language can profoundly impact the balance between insurer capabilities and individual rights. As legislators review current or proposed laws governing insurers' use of genetic information, they should consider whether the statute(s) accounts for a sufficient variety of insurer types, prohibits coverage differentiation even on the basis of actuarial justification, and requires individuals' informed consent before insurers or direct-to-consumer companies use their genetic data. Legislators should also examine the definition of genetic testing and the permissible range of insurer actions. In doing so, these state representatives are more likely to pass laws that align with their intent.

To learn more about state laws that govern insurance companies' use of genetic information, we invite you to visit our webpage: [Genetic Privacy in US Insurance and Criminal Law](#).